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# EXCITED DELIRIUM The Self-Fulfilling Prophecy of Police Brutality

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IN THEIR TIMELY and pressing piece, Arjun Byju and Phoebe Friesen explore the contentious diagnosis of excited delirium; a syndrome characterized by erratic, aggressive, and “delusional” behavior (2023). Overwhelmingly, this term is used when individuals come in contact with police and/or first responders. Although much attention has been given to debating whether or not this is a “real” diagnosis, the authors want to shift the focus to examining the work that excited delirium is doing in the world, as well as the underlying beliefs guiding it that are perpetuating substantial harm. Drawing significantly on the work of Ian Hacking (1998, 2006) and Sherene Razack (2015), Byju and Friesen explore what kinds of people are being made up by the label of “excited delirium” and the subsequent dehumanization that perpetuates violence towards those in this grouping.

Despite the lack of an agreed upon definition, excited delirium tends to frequently group together young Black men and other marginalized people of color, who have a psychiatric history, and those who have taken or are suspected to have taken drugs (Byju & Friezen, 2023). As this is “observed” when individuals come into contact with

law enforcement and/or first responders, those who are presumed to be suffering from excited delirium are typically met with physical restraints (chokeholds, hogties, etc.), forced injections of sedatives, and/or taser guns. The violent reactions are often justified not only by this “diagnosis,” but the employment of racial stereotypes; notably, the notion that Black bodies as being less susceptible to pain. As the authors aptly put it:

This kind of symptom language helps make up the novel category of a person who suffers from excited delirium: a crazed and inhumane monster, a golem that *must be* met with unrelenting and superordinate force, who is fated, regardless of interventions, to die a spontaneous death. This paradox, that those with excited delirium are supposedly both too strong *and* too weak, is at the core of the illness narrative. (2023)

It is evident that this label is reinforcing the combination of two harmful stereotypes: the notion that those suffering from severe mental illness occupy a space that is “less than fully human,” and racist stereotyping of Black bodies “super-seding human strength.” This perpetuates total dehumanization of the individual, where it is then justified to invoke brutal tactics, as well as signal

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to the greater community this is how individuals with mental illnesses and/or belonging to a particular racial group (chiefly, Black men), should be treated. This is despite the fact that those suffering from mental illnesses are significantly more likely to be victims of violent crimes, Black Americans are 3.2 times more likely to be murdered by police, and convergence of these factors (being Black and living with a mental illness) increases likelihood of such fatal interactions (Schwartz & Jahn 2020; Shadravan, Edwards, & Vinson 2021; Stuart, 2003).

Although I am in wholehearted support of their account, I take issue with the authors' conceptualization of excited delirium as non-falsifiable, as well as the notion that diseases, generally, are falsifiable. In an attempt to further criticize the utilization of excited delirium, Byju and Friesen call upon Karl Popper's falsifiability criterion as a measure for what constitutes as scientific; arguing that excited delirium is not a legitimate scientific concept as it is impossible to falsify. First, I am not convinced that excited delirium is in principle non-falsifiable; as the way in which it's conceptualized actually appears to be falsifiable. The authors point out that external factors, such as tasing and violent restraints, are clearly the causal explanation, which should in fact, lead one to reject the hypothesis of an excited delirium diagnosis. However, in practice, those who are seeking to exonerate the police/first responders responsible for these deaths, are not accepting this evidence leading them to reject the excited delirium diagnosis, but rather, are crafting an incorrect causal narrative. This is deeply problematic, but it does not necessarily render excited delirium as non-falsifiable. More broadly, I am concerned with the reliance on the falsifiability criterion, which has been widely criticized in the philosophy of science literature, thus, weakening the non-falsifiable argument (cf: Feyerabend, 1975; Hacking, 1983; Howson & Urbach, 1989; Jeffrey, 1975; Kuhn, 1962; Lakatos, 1970, 1980; Putnam, 1974; and Salmon, 1967).

Further, I want to call into question the notion that there are clear testable, causal stories when it comes to diseases; both somatic and psychological. According to Byju and Friesen, what sets excited delirium apart from other diagnoses is:

Regardless, disease frameworks typically employ measurable and causal processes. Some of these are well accepted (e.g., heart attacks are caused by cardiac ischemia which is caused by arterial atherosclerosis), but even when frequently challenged (e.g., depression is caused by low serotonin) (the monoamine hypothesis), these disease narratives include causal factors that may be tested, and ultimately refuted. (2023)

Although the authors do acknowledge there is space for diseases with unknown etiologies that should not be discounted as legitimate, such as irritable bowel syndrome, it is unclear what their stance is when it comes to causal processes for psychological disorders. From their perspective, it appears that what makes psychiatric disorders scientific is that they eventually bottom out at a causal process. Though, the classification of psychiatric diagnoses has a storied history, as well as a contentious present debate, when it comes to discerning what should or should not count as a clinical diagnosis. By insinuating that diseases must have a causal narrative, the authors risk implying that no psychiatric diagnoses are legitimate. Even if it is granted that psychiatric diagnoses can be placed in a similar category as diseases like irritable bowel syndrome, it is still unclear why excited delirium would be considered exempt; considering "diagnoses" such as drapetomania were once used in clinical settings to enforce oppressive structures.

Although I do not necessarily agree with the focus on the falsifiability and causal narrative criteria, I do believe the authors have best captured this phenomenon as a self-fulfilling prophecy. In line with Hacking's work (2006), Byju and Friesen discuss the work of looping effects in relation to excited delirium. They assert that "law enforcement and first responders are (un) wittingly facilitating the very supposed conditions suspected for excited delirium—they are reacting to the classification in ways that fit or justify the classification itself" (Byju & Friesen, 2023). From their account, it is evident that excited delirium is a byproduct of a looping effect—law enforcement, first responders, and those looking to shift the blame from those perpetuating the violence are reinforcing racial and sanist stereotypes to justify

further violent behavior. The upshot of moving away from falsifiability criterion and focusing more on looping effects is redirecting focus to the ways in which systematic racism and police brutality are not only being perpetuated, but also, where it can be intervened upon.

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